

Amar Raho Multi-Speciality Hospital

Emergency, Ambulance Dispatch & Code Blue Protocol

Policy Code	ARH-POL-05-EMERGENCY
Category	Emergency & ICU
Effective Date	January 2026
Version	1.0

Hospital Contact Information

- Emergency Helpline / Ambulance: +91 98765-AMAR-1 / 108
- General Reception: +91 98765-AMAR-2
- Email: contact@amarrahoahospital.com
- Website: www.amarrahoahospital.com
- Location: New Delhi, India — Block A: Outpatient & Administration; Block B: Emergency & Inpatient

1. Purpose and Scope

This policy establishes the hospital's 24/7 emergency access, ambulance dispatch process, triage priorities, emergency stabilization principles and Code Blue response.

2. Detailed Policy Clauses

- 2.1** The Emergency & Trauma Gate is located in Block B and operates 24 hours a day, 7 days a week.
- 2.2** Life-saving emergency stabilization shall not be delayed solely because an emergency patient cannot immediately provide an advance deposit. Financial formalities shall be addressed after or alongside stabilization as clinically appropriate.
- 2.3** Emergency triage prioritizes patients as Red (Immediate), Yellow (Urgent) and Green (Non-urgent). Triage priority may be revised as the patient's condition changes.
- 2.4** Red cases require immediate clinical assessment and intervention. Yellow cases require urgent assessment and treatment after immediate threats are addressed. Green cases are non-urgent and may wait according to clinical workload and triage reassessment.
- 2.5** Code Blue is the hospital's rapid-response escalation for a suspected or confirmed cardiopulmonary arrest or other situation requiring immediate resuscitation response, according to hospital clinical procedures.
- 2.6** On Code Blue activation, the designated response team shall respond rapidly, assess the patient, initiate appropriate resuscitation measures and coordinate escalation according to the patient's condition and authorized clinical protocols.
- 2.7** Ambulance dispatch is available 24/7 through +91 98765-AMAR-1 or 108. Dispatch timing and destination acceptance depend on vehicle availability, traffic, location, weather, clinical requirements and receiving-facility coordination.
- 2.8** The hospital has not specified a fixed ambulance reach radius in this policy; dispatch availability shall therefore be confirmed for each request rather than assuming a guaranteed geographic radius.
- 2.9** Ambulance requests should include caller name, callback number, pickup location, patient condition, approximate age, immediate hazards and destination when known.
- 2.10** Patients and attendants must follow emergency staff instructions and must not obstruct the Emergency Gate, resuscitation area or ambulance route.

3. Step-by-Step Patient / Attendant Procedure

Step 1. For an emergency, call +91 98765-AMAR-1 or 108, or proceed directly to the Emergency & Trauma Gate in Block B.

Step 2. Provide the patient's location and a concise description of the emergency to the dispatcher.

Step 3. At arrival, emergency staff perform triage and assign Red, Yellow or Green priority.

Step 4. Provide identification and insurance information when feasible, but do not delay immediate emergency stabilization to complete routine financial documentation.

Step 5. During a Code Blue, keep the area clear and follow staff directions so the response team can work without obstruction.

Step 6. After stabilization, complete registration, insurance/TPA and financial formalities as directed.

4. Escalation, Restrictions and Contact

4.1 Any immediate life-threatening deterioration inside the hospital should be reported at once to the nearest nurse or clinician so the emergency response can be activated.

4.2 For ambulance dispatch concerns, call +91 98765-AMAR-1 or 108.

4.3 For non-emergency administrative assistance, contact Reception at +91 98765-AMAR-2.

4.4 This policy does not replace clinician judgment, emergency medical protocols, or applicable law; clinical teams may take additional measures necessary for patient safety.

Document Control: This is an operational hospital policy. Applicable law, insurer terms, clinical judgment, or an individual patient-care plan may impose additional or stricter requirements.